Amerimed
WELLNESS INSTITUTE

Amerimed Wellness Institute
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LEGAL & COMPLIANCE · AMW-CMP-001

Compliance Program & Fraud, Waste and Abuse Policy

Anti-Kickback Statute, Stark Law, False Claims Act, billing integrity and exclusion screening

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Policy owner	Dr. Helen Tsai, JD, Chief Compliance & Ethics Officer	**Approved by**	Board of Directors, Audit & Compliance Committee
Applies to	All workforce members, medical staff, board members, contractors and vendors	**Classification**	Internal

1. Program structure

Amerimed's compliance program follows the HHS Office of Inspector General **General Compliance Program Guidance (November 2023)** and the seven elements of an effective compliance program:

Element	Amerimed implementation
1. Written policies and procedures	Code of Conduct; 140+ compliance policies on PolicyNet
2. Compliance officer and committee	Chief Compliance & Ethics Officer reports to the CEO and directly to the Board Audit & Compliance Committee
3. Training and education	Annual general training; role-based coding, billing and research training
4. Effective lines of communication	Integrity Line (24/7, anonymous); open-door policy
5. Enforcing standards	Consistent discipline; compliance performance in evaluations
6. Risk assessment, auditing and monitoring	Annual risk assessment and work plan; external coding audits
7. Responding to problems and corrective action	Investigations, root cause analysis, refunds and self-disclosure

2. Federal Anti-Kickback Statute (AKS)

It is a felony to knowingly and wilfully offer, pay, solicit or receive **anything of value** to induce or reward referrals of items or services payable by federal healthcare programmes (42 U.S.C. 1320a-7b(b)). - All arrangements with physicians and referral sources must fit a **safe harbour** where possible, be in writing, be at fair market value and be approved by Legal and Compliance. - Patient inducements (waivers of copays, free gifts) are restricted. Nominal-value items are limited to **USD 15 per item / USD 75 per year** per patient (per OIG policy). - Routine waiver of copays and deductibles is prohibited. Financial-need waivers follow AMW-PAT-003. - **California** (Business & Professions Code 650), **Oregon** and **Washington** have similar state laws that apply to all payers.

3. Physician Self-Referral Law (Stark)

Physicians may not refer Medicare patients for designated health services (DHS) to an entity with which they (or an immediate family member) have a financial relationship, unless an exception applies (42 U.S.C. 1395nn). This is a **strict liability** law, so intent does not matter. - Every physician arrangement is tracked in the **Contract Management System** and checked against an exception (e.g. employment, personal services, fair market value, non-monetary compensation up to the annual CMS limit). - Medical directorships require timesheets before payment. - Expired agreements are flagged 90 days before expiry. Payments stop on expiry unless the agreement is renewed.

4. False Claims Act (FCA)

Knowingly submitting false or fraudulent claims to federal programmes leads to treble damages plus per-claim penalties (adjusted annually; about USD 14,300 - 28,600 per claim). Examples include: - billing for services not rendered or not medically necessary - upcoding (e.g. billing a level-5 visit for a level-3 service) - unbundling - duplicate billing - billing for services by excluded or unlicensed providers - retaining overpayments for more than 60 days after identification

California False Claims Act (Government Code 12650) and the **Washington Medicaid Fraud False Claims Act** have similar provisions, including qui tam (whistleblower) rights.

5. Billing and coding integrity

- Coders hold AHIMA or AAPC credentials and are audited at least each quarter. The accuracy target is ≥ 95%.
- Clinical documentation must support the level and medical necessity of every service billed. Copy-forward and cloned notes are monitored.
- Teaching physician rules (42 CFR 415.172) apply to services involving residents.
- Overpayments are investigated promptly and repaid within **60 days** of identification (with a 6-year look-back).

6. Exclusion screening

All workforce members, medical staff, vendors and contractors are screened before engagement, and **monthly** after that, against: - the OIG List of Excluded Individuals/Entities (LEIE) - the GSA System for Award Management

(SAM) - the Medi-Cal Suspended & Ineligible Provider List, the Oregon Medicaid exclusion list and the Washington HCA exclusion list - state licensure boards

7. Conflicts of interest

Board members, officers, physicians in leadership roles, purchasing staff and researchers file **annual conflict-of-interest disclosures**. Research conflicts follow 42 CFR Part 50 Subpart F (PHS-funded research).

8. EMTALA compliance

Every person who comes to an Amerimed emergency department receives an appropriate **medical screening examination**, and stabilising treatment or an appropriate transfer, regardless of ability to pay or insurance status. Registration may not delay screening. Central log and on-call lists are maintained.

9. Reporting and investigations

- Report concerns to a supervisor, to the Compliance Office (compliance@amerimedwellness.example, 408-555-0190) or to the **Amerimed Integrity Line: 1-888-555-0198**.
- Investigations start within **5 business days**. Findings and corrective actions are reported to the Audit & Compliance Committee.
- Where appropriate, Amerimed uses the **OIG Self-Disclosure Protocol** or the **CMS Voluntary Self-Referral Disclosure Protocol (SRDP)**.

10. Non-retaliation

No one will be retaliated against for reporting in good faith. This protection includes qui tam relators under 31 U.S.C. 3730(h). Retaliation is grounds for termination.

11. Record retention

Billing and medical records are kept for at least **10 years** (or longer if state law, a minor's age of majority plus the statutory period, or litigation holds require). Medicare Advantage records are kept for 10 years under 42 CFR 422.504.

Amerimed Wellness Institute is a fictional healthcare organisation created for AI training and demonstration. It does not provide medical care. All names, facilities, addresses and identifiers are invented. Nothing in this document is medical or legal advice.